

Anxiety-related and depression-related visits to primary care before, during, and after the COVID-19 pandemic



The long-term mental health burden associated with COVID-19 has been documented across multiple populations,¹ including individuals in need of care with substantial and lasting reductions in quality of life and mental health.² Analyses from the International Consortium of Primary Care Big Data Researchers (INTRePID) have shown that primary care systems worldwide underwent highly variable and often abrupt shifts in mental health-care utilisation during the early COVID-19 pandemic period, but these analyses involved the initial pandemic period or short follow-up windows, leaving the longer-term impacts on primary care systems in the post-pandemic period largely unknown. These contributions include a cross-national analysis across six countries³ that found substantial early pandemic disruptions in primary care use and an interrupted time-series study across nine countries⁴ that found immediate pandemic-related increases in anxiety-related and depression-related visits. The Article by Alison Flehr and colleagues⁵ builds on this emerging body of work by providing a comprehensive, post-pandemic comparative analysis of anxiety-related and depression-related primary care visits across 11 countries. This cross-sectional study offers one of the clearest international perspectives to date on how primary care has absorbed the mental health burden of the COVID-19 pandemic. Drawing on 1 059 101 897 primary care visits—of which 49 022 341 (4.6%) were attributed to anxiety or depression—the authors provide a comparative analysis spanning the full pre-pandemic, pandemic, and recovery periods. This three-phase structure enables a nuanced examination of how patterns evolved over time, showing that anxiety-related and depression-related consultations rose sharply during the pandemic and remained elevated in several countries, underscoring the sustained mental health needs in the aftermath of the COVID-19 pandemic that primary care systems continue to absorb. A notable strength of the study is its use of a psychosocial life-stage framework (emerging adulthood: age 20–29 years; established adulthood: 30–49 years; middle adulthood: 50–69 years; and late adulthood: 70 years and older)⁶ and sex stratification, which reveal demographic patterns that routine surveillance has largely obscured. Women consistently had higher consultation rates, individuals in emerging adulthood

carried the highest proportional burden, and those in middle and late adulthood had the most pronounced increases in consultation rates during the pandemic compared with those during the pre-pandemic period. Equally notable are the distinct national footprints of mental health-care use, shaped by differences in health-system organisation, pandemic policies, and sociocultural context. A clear implication of these findings is that mental health care in primary care does not follow a universal trajectory; accordingly, policy responses must be tailored to local systems, rather than being anchored in assumptions of homogeneity. Nevertheless, because some datasets analysed in the study were regional, their generalisability to national populations may be constrained; hence, findings from these settings should be interpreted with caution.

Moreover, important questions remain. The study cannot determine which specific structural or sociocultural factors drive wide cross-country differences or whether increased utilisation reflects rising clinical needs, changing diagnostic practices, or expanded access. In addition, whether these patterns translate into improved patient outcomes remains unknown. Addressing these gaps will be essential for building resilient primary care systems capable of responding to future crises, reflecting wider concerns about system capacity highlighted in global mental health policy reports.⁷ What this study ultimately demonstrates is that primary care remains the backbone of mental health-care delivery and that its role has intensified in the aftermath of COVID-19. Strengthening primary care infrastructure, integrating mental health support, and safeguarding workforce capacity must therefore be central components of future preparedness strategies. Likewise, WHO's Mental Health Atlas (2024) emphasises the persistent global shortfalls in mental health system investment and workforce availability.⁸

I declare no competing interests. During the preparation of this work, the author used ChatGPT-5.2 in order to perform language editing (grammar and spelling). After using this tool, the author reviewed and edited the content as needed and takes full responsibility for the content of the publication.

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Lancet Prim Care 2025;
1: 100090

Published Online December 17,
2025

<https://doi.org/10.1016/j.lanprc.2025.100090>

See [Articles](https://doi.org/10.1016/j.lanprc.2025.100070) <https://doi.org/10.1016/j.lanprc.2025.100070>

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